

Far From Help

Trained-hands lambing procedures for when the vet is ninety minutes away

The main handbook marks these procedures “trained hands only”. On a property ~1.5 hours from veterinary help, they are the farmer’s to learn — as veterinary and agricultural extension services teach them. This supplement collects the published versions: to learn before lambing, and to have open beside you, vet on speakerphone, if the night comes.

The rules of engagement

Distance changes what you do while help comes. It does not remove the phone call.

- **Still ring first — earlier, in fact.** With 1.5 h of travel, “watch for 30 minutes” becomes “**ring at 10–15 minutes and watch while you talk.**” Every procedure here can be phone- or video-guided.
- **Get shown beforehand.** Book a pre-lambing visit or workshop with your clinic: stomach-tubing, the IP injection site, and an internal exam demonstration take an hour to learn with a teacher. That hour converts everything below from “first time, alone, 2 a.m.” to “revision”.
- **Know Plan B: load and drive.** A ewe in trouble travels better than you'd think — bedded trailer or covered crate, lamb with her if born. Driving toward the vet (or meeting halfway) is often faster than waiting. Decide early.
- **Every procedure has printed stop conditions.** Stopping and driving is never a failure.

The remote playbook — set up before lambing

DO NOW	WHY
Program every clinic within range + after-hours numbers; print them in the shed	Reachability is your stated risk — hold more than one number
Ask each clinic: after-hours protocol, video-call willingness, travel time, meet-halfway option	Turns a 90-minute problem into a 45-minute one
Book the pre-lambing training visit ; ask about a “ distance pack ” (oral energy drench, SC calcium + training, pain relief/antibiotic cover with defined instructions)	Normal practice for remote clients — but it must be arranged in advance, with them
Test a video call (FaceTime/WhatsApp) from the lambing shed — check reception there	A vet who can see the presentation can guide precisely
Assemble the extended kit (Section 5) and stage the transport option, bedded	The extra items are cheap; the drive isn't
Write the property's RAPID number/directions where anyone can read them to a vet	Saves minutes when they matter

DURING ANY EMERGENCY IN THIS SUPPLEMENT

Phone on speaker, in your chest pocket, vet on the line, and work together. Only if truly unreachable do these pages stand alone.

Internal exam & correcting a malpresentation

The skill is identification before action: trace every limb to its body, cup every hoof, pull only what you have identified — and only with her strains.

Sources: NADIS Lambing Part 1; University of Wisconsin Extension Visual Lambing Guide (video — watch every episode before lambing); Sheep 201.

Before your hand goes anywhere near her

- 1 | **Ring the vet / start the video call.** Describe or show what's presenting.
- 2 | Wash hands and arms in warm diluted disinfectant; nails short. Long glove on.
- 3 | **Lubricant — more than you think you need.** Re-lube at every re-entry.
- 4 | Restrain: cast her on her side — for a leg-back, **missing-leg side uppermost** (NADIS). To repel a head, easier standing or with hindquarters raised on a bale.
- 5 | Hand in a cone, enter slowly, **work between contractions; pause when she strains.**

IDENTIFY BEFORE YOU PULL — ANYTHING

Front leg: fetlock and knee bend the **same** way. Hind leg: fetlock and hock bend **opposite** ways. A tail near hooves = hind end. With twins, follow each leg to its shoulder or hip and confirm both legs you hold belong to the **same lamb**.

Leg back — the most common fixable problem

- 1 | Gently push the lamb back toward the ewe (between strains) to make room.
- 2 | Follow the missing shoulder → knee → cannon → fetlock.
- 3 | **Cup the hoof in your palm** — this shields the uterus from the sharp toe. The single most important detail.
- 4 | Flex the knee; bring the cupped hoof up and over the pelvic brim in one smooth motion.
- 5 | Both legs + head now in the diver position → deliver with the traction rules below.

Both front legs back — “hung lamb” (head out, no legs)

- 1 | Keep the head moist; membranes off the nose, throughout.
- 2 | **Gently repel the head back inside** between strains (standing ewe / rear raised helps). It feels wrong; it is the correct move.
- 3 | Retrieve each leg exactly as for leg-back — cup every hoof.
- 4 | If the head slips back, a **lambing snare behind the ears and through the mouth** (never around the jaw alone) holds it in position while you deliver.
- 5 | Cannot repel the head within a few minutes → **stop: load and drive**. A swollen head can still deliver alive at the clinic.

Head back (two legs, no head) **PHONE-GUIDED ONLY**

The hardest common correction — the head flips back easily in a small space. Published sequence: repel the lamb; follow the neck to the head; **cup the muzzle in your hand** (never hook the eye sockets); bring it round; snare it if it won't stay. **Two failed attempts = load and drive.**

Hind legs first (soles up) — deliver it, promptly

Do not convert it. Once the hips engage, the cord pinches and the lamb will try to breathe inside. Confirm both hooves are hind feet of the same lamb, then pull both legs **steadily down toward her hocks with her strains, briskly through the ribcage stage** (Wisconsin: “quick action is critical once the umbilical cord snaps”). Clear the airway instantly; expect to resuscitate.

Breech (tail only) **PHONE-GUIDED ONLY**

Repel the rump; find a hock; cup the hoof; flex it up over the brim; repeat; deliver as hind-first. Farmers do this — with room, patience and a well-lubed arm. **Two failed attempts = load and drive.**

Two lambs at once **PHONE-GUIDED ONLY**

Repel the less-advanced lamb; deliver the engaged one. The whole skill is proving which legs belong to which lamb before pulling. Can't sort the tangle in a few minutes → load and drive.

TRACTION — THE UNIVERSAL RULES (NADIS)

Pull **only during her strains**, only on two legs + head in position (or two hind legs). Direction: a **downward arc toward her hocks** — “down and around, never straight out.” A normal assisted delivery takes **10–30 seconds** of steady-arm traction. Never ropes to fenceposts; never more force than your arms.

Stop = load and drive when: the canal is dry/swollen beyond what lube fixes · two well-applied strains move nothing · you can't identify what you feel · her pain escalates.

After any assisted delivery

- Airway first: clear, rub hard, straw-tickle a nostril. Leave the cord intact 1–2 min if unbroken.
- **Check for another lamb** — clean re-lubed glove, or ballot the belly just in front of the udder.
- Iodine the navel; colostrum early — an assisted lamb is a slow lamb.
- Tell the vet what you did; assisted ewes commonly warrant antibiotic cover (their call, their supply). Watch her 48 h for tears, infection, retained placenta.

Stomach-tubing a lamb

For the lamb that can swallow but won't suck. Measured, slow, and checked — a tube at the right length in the food-pipe cannot go wrong; a tube in the windpipe kills.

Sources: OMAFRA (Ontario) Care of the newborn lamb; OSU Sheep Team; Cornell Cooperative Extension.

USE / NEVER

Use when: the lamb can hold its head up and swallow, is $\geq 37^\circ\text{C}$ (warm it first if not), and needs colostrum it won't take from teat or bottle.

Never tube: a lamb that can't hold its head up or swallow · a lamb $< 37^\circ\text{C}$ older than ~5 h — that lamb needs the IP injection first (Procedure 3): *“never stomach tube before an IP glucose injection — milk could kill the lamb”* (SRUC).

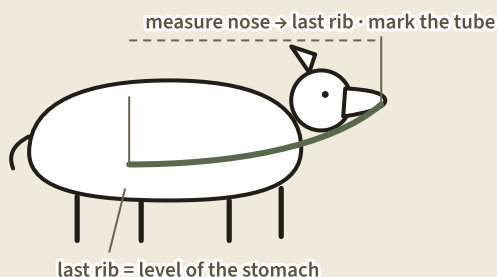


FIG. 1 Measure before anything else. Nose to last rib, tape-mark that length on the tube — it is further in than feels right (most of the tube).

- 1 | **Measure and mark:** tube held alongside the lamb, nose → last rib.
- 2 | Sit; lamb upright on your lap, head in a natural nose-forward position (not stretched up).
- 3 | Wet the tip with colostrum. Insert at the **side of the mouth**, over the tongue, aiming slightly to the **lamb's left** (the food-pipe lies left of and behind the windpipe).
- 4 | Advance slowly. **Right = it slides easily**, the lamb may chew and swallow it, and with a finger on the throat you can feel **two tubes** (windpipe + yours).
Wrong = resistance, coughing, struggling → withdraw completely, start again.
- 5 | Extra check: ear to the tube's end — airflow or crying = windpipe, withdraw. Silence or stomach gurgle = correct.

- 6 | Syringe on, **plunger out**: let warm colostrum run by gravity (or push very gently). **50 ml per kg over ~5 minutes**. Never fast.
- 7 | **Crimp the tube firmly, then withdraw** in one smooth motion (no drips into the airway on the way out).
- 8 | Warm dry pen; recheck 30–60 min; repeat toward ~200 ml/kg by 24 h if it still won't suck.

Intraperitoneal (IP) glucose injection

The cold-lamb rescue — the reason the handbook says “do not warm first”. Farmer-administered across the UK, Ireland and NZ; the abdominal cavity absorbs glucose safely, which is why extension services teach it.

Sources: Farm Advisory Service Scotland; SRUC; OSU; Cornell; original evidence Eales & Small 1982, Vet Record.

USE ONLY WHEN ALL THREE ARE TRUE

Lamb **below 37 °C** · **older than ~5 hours** · **cannot swallow / hold its head up**. A lamb that can swallow gets warmed and tubed instead — never both orders reversed.

Prepare — shop before lambing

20% glucose/dextrose solution (vet or rural store — ask for it in your pre-lambing “distance pack”), or make it: **20 g glucose powder in hot pre-boiled water made up to 50 ml (40%)**, then **diluted half-and-half with warm pre-boiled water** → ~20%, ~50 ml, warm-on-the-wrist (SRUC). **Dose 10 ml/kg** — a 4–5 kg lamb takes the full ~45–50 ml. 50–60 ml syringe; new sterile 1-inch (25 mm) needle.

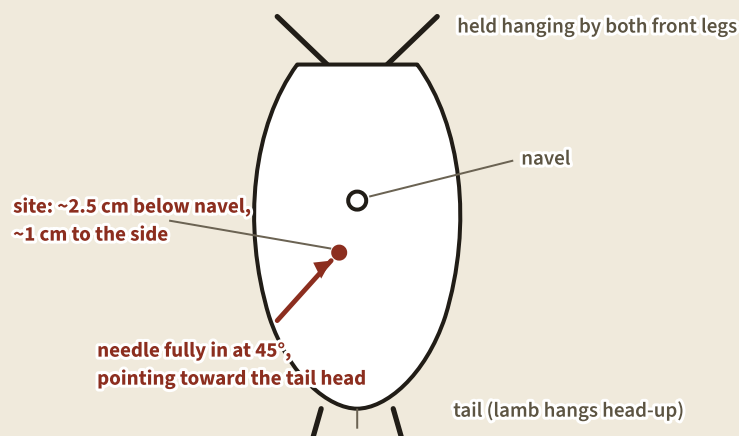


FIG. 2 The site — ~2.5 cm (1") below and ~1 cm (½") beside the navel, needle at 45° toward the tail head, lamb held vertically so the organs fall clear (FAS Scotland / SRUC / Cornell).

- 1 | Ring the vet (speaker, pocket). Draw up the **warmed** solution.
- 2 | **Hold the lamb hanging by both front legs** (helper ideal; alone, forelegs in one hand, lamb's back against your legs).

- 3 | Site as diagrammed. Insert the **full needle at 45°, angled toward the tail head** — two faint pops (skin, then wall).
- 4 | **Inject the full dose slowly and steadily.**
- 5 | Withdraw; rub; iodine the site.
- 6 | **Now warm:** warm-air box ~35–40 °C, check every 20–30 min; recheck lamb temperature at 30 min. Expect it standing within 30–60 minutes.
- 7 | Once ≥37 °C and swallowing: **tube 50 ml/kg colostrum** (Procedure 2), then re-mother.

DO NOT

Inject a lamb that can swallow (tube it) · use cold solution (deepens the hypothermia) · reuse needles between lambs · inject at or forward of the navel itself.

Replacing a bearing & fitting a retainer

NZ clinics publish this because bearings happen far from help. The vet-preferred version uses an epidural so she can't strain against you — so the call still comes first: they may say "bring her in", or talk you through this.

Sources: *Farmers Weekly* step-by-step; Northland Veterinary Group (NZ); Agriculture Victoria; OSU.

CHECK FIRST — STOP CONDITIONS

Tissue torn, black or cold → do not replace; ring/drive — a vet must judge whether it's salvageable. Signs she is **actually lambing** (water bag, feet or tail palpable through the cervix behind the mass) → this is a lambing emergency now; ring immediately.

- 1 | Two people if possible: one holds her standing against a rail; **raising her hindquarters** (bale under back legs, or rear held up wheelbarrow-style) makes everything easier.
- 2 | **Clean:** plenty of clean warm water with mild antiseptic (dilute iodine, "weak-tea colour" — Northland Vets). All dirt and bedding off. No scrubbing.
- 3 | **Lubricate** the whole surface generously.
- 4 | **Replace:** first **lift the mass up toward her tail** — this un-kinks the bladder; she'll often pass urine and the ball shrinks. Then, with the **flat of the hand** or a **closed fist** — **never fingertips** — slow, steady, patient pressure. **Hold when she strains; gain between strains.** It returns gradually, then all at once; follow it in so the tip is fully un-inverted.
- 5 | **Retain:** bearing spoon/T-piece — tongue in the vagina, wings tied firmly to flank wool with the tapes — or a prolapse harness per its instructions. **Buy one now;** improvising at midnight is miserable.
- 6 | **Aftercare:** she warrants pain relief and usually antibiotic cover — vet-supplied in NZ: ring, report, arrange (ask at the pre-lambing visit whether they'll pre-supply for your distance). Check the retainer twice daily. **She can lamb past a spoon** (it pushes out) — but watch her closely at lambing; a harness must be released. **Mark her; bearings recur — don't breed her again.**

Extended kit · what stays vet-only

KIT ADDITIONS FOR THE REMOTE PROPERTY

ITEM	FOR	NOTE
20% glucose/dextrose (or powder + jug)	IP injection	Vet “distance pack” or rural store
50–60 ml syringes ×3 · sterile 1” needles ×6	IP injection	Rural store
Bearing spoon and prolapse harness	Bearing	Buy before lambing
Lambing snare	Holding a corrected head	Watch the Wisconsin video on its use first
Second bottle of obstetric lube	Everything	You cannot have too much
Printed §§2–4 of this supplement, laminated	The paddock has no wifi	—
Bedded trailer / ute crate, staged	Plan B: load and drive	Test-load a calm sheep once

THE HIGHEST-VALUE HOUR OF YOUR PREPARATION

Ask your clinic for a **pre-lambing visit + “distance pack”**: tubing and IP demonstration, oral energy drench, subcutaneous calcium (with training), pain relief and antibiotic cover with written instructions for defined situations. Rural NZ practices do this for known clients 1.5 h out — but it must be arranged with them, in advance.

STILL GENUINELY VET-ONLY — AND PLAN B FOR EACH

PROBLEM	WHY VET-ONLY	YOUR MOVE
Uterine prolapse (huge mass, buttons)	Epidural + technique; forcing can rupture an artery	Ring; keep her still, mass in clean wet towel off the ground; load and drive is usually right — she travels lying, mass supported
Ringwomb	Usually caesarean	Ring early — it declares itself over hours, so you have driving time
Oversized lamb truly stuck after correct position + failed gentle traction	May need caesarean	Load and drive — don’t spend an hour trying
Uncontrolled fresh bleeding	Surgical	Ring; still and calm; drive if told
Down ewe (milk fever / sleepy sickness)	IV calcium is vet-only; SC calcium + oral drench are farmer tasks with vet training/supply	Arrange training + supply at the pre-lambing visit
Caesarean · embryotomy	—	The drive is the treatment

What the wider evidence says about your ewes

A targeted literature search (including the German breed name, Walliser Schwarznasenschaf) confirms: no peer-reviewed lambing or dystocia dataset exists for the Valais Blacknose. “Easy lambing” remains a breeder claim. The transferable science:

- **Dystocia tracks lamb birthweight above all else** — assistance is rarest near ~3.5 kg and climbs steeply past ~4.5 kg; well-fed singles are the classic oversize risk (Smith 1977; Dorset, Merino and NZ Romney studies). *Practical: your singles-bearing ewe on lush spring feed is the likeliest assisted birth — don’t over-feed her final month, and watch her closest.*
- **Ewe frame helps.** VB ewes are large (70–90 kg; CABI breed data), and pelvis-to-lamb ratio is what matters (rare-breeds pelvimetry, Richardson et al.). *Practical: a first-timer lambing a big single is the higher-risk combination in your flock.*
- **Failure of cervical dilation is the most common maternal cause of dystocia** (302-case hospital series, 2022). *Practical: hours of nesting with no water bag is the quiet emergency — with your travel time, ring at hour 4–6, not hour 8.*
- The breed appears in Swiss population-genetics work only (Signer-Hasler et al. 2019 — inbreeding/selection across eight Swiss breeds). Horns in both sexes are soft buds at birth — irrelevant to dystocia. The fleece is not: crutch, dag, clear the udder pre-lambing.

SOURCES

1. NADIS — *Lambing Part 1: The Basics*. nadis.org.uk
2. University of Wisconsin–Madison Extension — *The Visual Lambing Guide* (video series). livestock.extension.wisc.edu/visual-lambing-guide
3. OMAFRA / Ontario — *Care of the newborn lamb*. ontario.ca/page/care-newborn-lamb
4. OSU Sheep Team — *Tube feeding; IP injection of dextrose*. u.osu.edu/sheep
5. Cornell Cooperative Extension — *Tube feeding and IP injections*.
6. Farm Advisory Service Scotland — *Treating hypothermia in newborn lambs*. fas.scot
7. SRUC — *Treating cold lambs with an IP injection*. sruc.ac.uk
8. Eales FA, Small J (1982). *Resuscitation of hypothermic lambs*. *Vet Record* (PMID 7186698).
9. Farmers Weekly — *Step-by-step: vaginal prolapse in sheep*. fwi.co.uk
10. Northland Veterinary Group (NZ) — *Bearings*. northvets.co.nz · Agriculture Victoria — *The prolapsing ewe*.

11. Dystocia evidence: Smith 1977 (PMID 558970) · George 1976 (PMID 1016153) · NZ Romney studies (PMID 270640, 292902) · rare-breeds pelvimetry (PMID 6495569) · ovine dystocia case series 2022 (PMC9297778) · Signer-Hasler 2019 (PMID 31365135).

Compiled August 2026 for a property ~1.5 h from veterinary help. These are the published farmer-education versions of each technique; hands-on instruction beforehand remains the strong recommendation, and a phone or video call to the vet remains step 1 of every procedure.